REQUEST FOR INFORMATION (RFI)

Enterprise Software Services (ESS) Next

1. BACKGROUND

The Defense Health Agency (DHA) Program Executive Office for Defense Healthcare Management Systems (PEO DHMS) is conducting market research to inform acquisition planning for the ESS Next requirement. ESS Next provides infrastructure and environments to support development, integration, and testing for DHMS stakeholders including DoD Healthcare Management Systems Modernization (DHMSM), Joint Operational Medicine Information Systems (JOMIS), Enterprise Intelligence and Data Solutions (EIDS), and the Federal Electronic Health Record Modernization (FEHRM) programs and their contractors. The budget environment is extremely constrained, and PEO DHMS is seeking innovative solutions to efficiently and effectively meet PEO DHMS high-level objectives.

2. SCOPE OF REQUIREMENT

The ESS Next will meet the following high-level objectives:

- Optimized and Consolidated Infrastructure and Facilities
- Modernized, Secure, and Operationally Aligned Service Delivery
- Integrated and Efficient Testing and Validation Services
- Enterprise Tooling to Enable Product Delivery
- A Transparent and Cost-Effective Platform Service
- Seamless Operational Continuity and User Support

3. CONTRACT INFORMATION

- **Contract Type:** Firm Fixed Price (FFP)
- **Period of Performance**: The PoP shall be for one (1) Base Year of 12 months and four (4) 12-month option periods.
- **Location:** National Capital Region (within 30 miles of Arlington, VA)

4. INFORMATION REQUESTED

This RFI seeks feedback from industry on the attached Draft Statement of Objectives (SOO) and Current State Analysis to inform acquisition strategy decisions. Responses will help determine the most appropriate acquisition approach and identify any requirement modifications necessary to enable effective competition.

A. Requirements Assessment

1. What clarifications or additional information do you require to the draft SOO and Current State Analysis to propose a cost effective and innovative solution that will meet DHMS' requirements for ESS?
    a. Are there any aspects of the draft SOO you believe may present implementation challenges or warrant further consideration? Are the objectives defined well enough for bidders to draft scope which will lead to efficiencies and effectively meet DHMS requirements? Please include any recommendations for clarification, adjustments, or alternative approaches to enhance feasibility and effectiveness.
    b. What additional information should we include in the Current State Analysis for bidders to propose cost-effective and innovative solutions to meet DHMS needs and modernize ESS?
2. How would you achieve cost savings through innovation?
3. How long of a transition period is needed for the current, incumbent, vendor to provide transition-out support, based on the information provided in the Current State Analysis?
4. Based on the information provided in this RFI, which outcomes are the most impactful cost drivers for ESS, and how can we loosen or modify those outcomes to enable bidders to propose efficient and cost effective solutions?
5. What contract types and strategies would you recommend to provide an incentive for bidders to optimize the ESS environment, achieve savings, and to share those savings with the government (e.g., FFP with successive price reductions)?
6. What pricing models exist under an FFP contract that accommodate both planned capacity reduction over the period of performance and short-term demand variability (e.g., tokens)? Please describe any proposed unit of measure, how cost transparency would be achieved, and provide a specific example of this approach implemented on a comparable government contract.
7. Organizational Model for Test Infrastructure & Test Support
    a. Describe how your organization structures the relationship between test infrastructure and test support services (e.g., test engineering, automation, functional developmental testing, integration testing, operational testing)
    b. What roles and responsibilities are typically client/government-owned vs vendor-owned when operating enterprise test environments?
    c. Describe the staffing models used to support large-scale enterprise test environments, including how testing personnel interact with software development teams, systems integrators, and operational stakeholders
8. Test Infrastructure Architecture & Environment Strategy
    a. Describe typical test infrastructure architectures used to support multi-vendor healthcare Information Technology (IT) systems, including approaches for integrating systems hosted across different network domains (e.g., internal enterprise networks, cloud-hosted environments, vendor environments).
    b. Describe approaches used to maintain representative test data sets, including strategies for de-identification of clinical data and data refresh cycles.
    c. What tools or platforms are typically used to provision and manage test infrastructure, including infrastructure-as-code, environment orchestration, and observability tooling?
    d. Describe the test infrastructure architectures you have used to support multi-vendor healthcare IT systems, including approaches for integrating systems

hosted across different network domains (e.g., on-premises enterprise networks, cloud-hosted environments, and vendor-managed environments). Include any constraints or tradeoffs specific to DoD classified or restricted network environments.

  e. How do you design and maintain development, integration, staging, and test environments to ensure configuration parity with production systems, prevent environment drift, and manage promotion between environments when multiple development teams share the same infrastructure?
  f. What tools and platforms have you used to provision and manage test infrastructure, including infrastructure-as-code, environment orchestration, and observability tooling? Describe the tradeoffs between approaches evaluated and the basis for your selections.

9. Risk Mitigation Through Testing

  a. How do healthcare organizations and software vendors use testing to mitigate operational, cybersecurity, and integration risk prior to deployment?
  b. What risk-based testing approaches are used to prioritize testing efforts when multiple software releases or system changes occur simultaneously?
  c. How are performance, scalability, and resilience risks evaluated during testing?
  d. What methods are used to identify and manage regression risks when new capabilities are introduced into existing healthcare IT environments?
  e. Describe approaches you have used to validate interoperability between independently developed systems, specifically testing of APIs, message interfaces, and data exchanges in a multi-vendor healthcare IT environment. Identify the standards and tools used and any gaps encountered.

10. Software Lifecycle Testing Practices

  a. Describe how testing activities are integrated across the software development lifecycle, including development testing, integration testing, independent verification and validation (IV&V), and operational testing.
  b. How are automated testing capabilities used throughout the software lifecycle, including unit, integration, regression, and performance testing?
  c. Describe best practices used to coordinate testing across multiple vendors contributing to a shared enterprise platform.
  d. Describe how DevSecOps pipelines are used to enforce quality gates and automated testing prior to software promotion between environments. What is the boundary between pipeline tooling that is infrastructure-provider-owned versus development-team-owned, and how is that boundary defined and maintained?

11. Industry Standards & Best Practices

  a. What industry standards or frameworks are commonly used to guide testing practices for custom healthcare software development?
  b. What tools and methodologies are commonly used to automate testing for healthcare IT systems, particularly for clinical workflows, interoperability validation, and security testing?
  c. Describe emerging trends or innovative approaches in healthcare software testing, including artificial intelligence-assisted testing, synthetic data generation, or automated validation frameworks.

12. Cost Transparency & Resource Management

a. How do organizations structure test infrastructure and test support services to maximize cost transparency and traceability?
b. What cost drivers most significantly impact enterprise test environments, including infrastructure, data management, automation tooling, and workforce support?
c. How do organizations measure the cost efficiency of test infrastructure investments, particularly when supporting multiple software vendors and development teams?
d. What contracting or service delivery models are commonly used to provide scalable test support services?

13. Scalability & Demand Management

a. How do healthcare systems scale test infrastructure and testing support resources to accommodate varying volumes of software releases, patches, and capability upgrades?
b. Describe approaches used to prioritize testing activities when competing demands exist across multiple programs or vendors.
c. How are test environments dynamically provisioned or expanded to support surge development activity or large software releases?
d. What role does cloud infrastructure play in enabling scalable testing capabilities?

14. Performance Measurement & Success Metrics

a. What metrics are commonly used to measure the effectiveness of enterprise test infrastructure and testing services?
b. How do organizations measure the impact of testing on system quality, reliability, and operational readiness?
c. What indicators are used to assess the maturity of testing capabilities within large healthcare IT environments?
d. Describe methods used to track defect trends, test coverage, and release readiness across large-scale healthcare IT systems.

5. RESPONSE REQUIREMENTS

- **Response Deadline:** 15 calendar days from RFI publication
- **Format:** Electronic submission preferred
- **Points of Contact:** Krystal Mason, krystal.m.mason2.civ@health.mil;

  J. R. Oliver, j.r.oliver.civ@health.mil;

Instructions for Submission

RFI respondents may respond to all or some of the questions above. Responses and comments may be utilized in the development of additions or modifications to future PEO DHMS Draft Request for Proposals (RFP). Please keep responses to the above questions to no more than 10 pages including any appendices, exhibits, diagrams, or supplemental materials. Submissions exceeding the specified page limit may be deemed non-compliant and may not be reviewed.

PEO DHMS will not respond to questions to this RFI.

PEO DHMS requests that responses to the RFI be submitted to the POC identified below by 9 April 2026.

RFI responses should be submitted to the following:

Krystal Mason, krystal.m.mason2.civ@health.mil

No verbal responses will be accepted.

Proprietary information may be submitted; however, RFI respondents are responsible for adequately marking proprietary, restricted or competition sensitive information contained in their response. All submissions will be protected from disclosure outside of PEO DHMS.

In addition, RFI respondents should be aware that PEO DHMS may utilize contractor support personnel from the below listed companies (under existing contracts) to review responses and information submitted. These companies and individual employees are bound contractually by Organizational Conflict of Interest and disclosure clauses with respect to proprietary information, and they will take all reasonable action necessary to preclude unauthorized use or disclosure of an RFI respondent's proprietary data. RFI responses MUST clearly state whether permission is granted allowing the program office support contractors identified below access to any proprietary information.

- Boston Consulting Group
- Agensys Corporation
- Monterey Consultants
- Technology Experts, Inc.
- Optima Global Solutions, Inc.

This RFI is not a solicitation. This RFI is for planning purposes only. It does not constitute an RFP or a promise to issue an RFP in the future. This RFI does not commit the Government to contract for any supply or service whatsoever. Further, PEO DHMS is not seeking proposals at this time and will not accept unsolicited proposals. Respondents are advised that the Government will not pay for any information or administrative costs incurred in response to this RFI. All costs associated with responding to this RFI will be solely at the responding party's expense. Participation is not mandatory or required; participation or response to this RFI is not a prerequisite for any future procurement activities.

Attachments:
1 – Draft SOO
2 – Draft Current State Analysis